

Oscar Health, Inc.

Already Paid for a Repricing That Has Not Happened Yet: The Subsidy Cliff Is the Whole Call; Underweight, Dec-27 PT \$15.50

OSCR, NYSE | Healthcare, Managed Care

UNDERWEIGHT

Price (27 Aug 2025): **\$17.02**

Price Target (Dec-27): **\$15.50**

INVESTMENT THESIS

We initiate on Oscar Health at Underweight with a Dec-27 target of \$15.50, 8.9% below the \$17.02 close of 27 August. This is not a business that is failing. It is a business whose 2026 recovery is already in the price and whose single largest variable, the scheduled 31 December expiry of the enhanced ACA premium tax credits, remains unlegislated with open enrolment ten weeks away. Oscar cut full-year guidance on 22 July from earnings from operations of \$225m to \$275m to a loss of \$200m to \$300m, a swing of roughly \$500m at the midpoints, and reported a Q2 medical loss ratio of 91.1% against 79.0% a year earlier. The shares are nonetheless 32% above their July low and trade at 3.79x June book value against Centene at 0.52x and Molina at 2.05x.

The quality of the miss matters more than its size. CFO Scott Blackley attributed the quarter to "an incremental \$316 million increase to our risk adjustment payable for 2025", which is 11.3 points of Q2 premium and therefore nearly all of the 12.1 point MLR deterioration, while stating that "second quarter utilization moderated meaningfully as compared to the first quarter". This is a risk-pool composition problem, not a care-consumption problem, and it is repairable through price. Our objection is arithmetic, not directional: even crediting a full return to a 3.5% operating margin by 2027, our three anchors blend to \$15.50. The market is paying today for delivery that management has never yet achieved and has not quantified.

- Q2's 91.1% MLR is essentially one line item: a \$316m risk adjustment true-up worth 11.3pp of premium. Underlying utilisation moderated through the quarter.
- 2026 revenue falls, not rises, in our base case: pricing up roughly 24% against membership down roughly 25% gives \$11.25bn, a 7% decline.
- At 3.79x book and 0.36x FY25E revenue the stock already discounts the recovery. Enacted subsidy extension is the one event that flips us.

COMPANY OVERVIEW

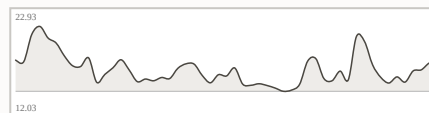
Oscar Health is the largest technology-led health insurer built specifically for the ACA individual exchanges, operating individual and family plans, small group and ICHRA products, plus the +Oscar technology platform sold to third-party payers and providers. Effectuated membership was 2,027,148 at 30 June 2025, up 28% year on year, of which 2,017,058 sat in individual and small group. Mark Bertolini, formerly of Aetna, has been CEO since April 2024.

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PRICE PERFORMANCE



Weekly closes (\$), 28 Aug 2024 to 27 Aug 2025. 52-week intraday range \$11.20 to \$23.79; last close \$17.02.

COMPANY DATA

Shares O/S (mm, 30 Jun 25)	258.2
Market Cap (\$bn)	4.40
Total Debt (\$mm)	357
Cash & Investments (\$bn)	5.38
Enterprise Value	n.m.
52-Week Range	\$11.20 - \$23.79
52-Week Price Change	-1.4%
Avg. Daily Volume (Aug-25, mm)	19.2
Short Interest (% of S/O)	-
Beta (5Y)	-
P/E (TTM, GAAP)	n.m.
Price / FY26E EPS	23.6x
P/B (BVPS \$4.49)	3.79x
Price / FY25E Revenue	0.36x
Dividend Yield	None
Fiscal Year End	December

KEY METRICS / ESTIMATES

(\$mm, exc. EPS)	FY23A	FY24A	FY25E	FY26E
Total revenue	5,863	9,178	12,100	11,250
% growth	48%	57%	32%	-7%
Medical loss ratio	81.6%	81.7%	86.5%	81.0%
SG&A ratio	24.3%	19.1%	17.4%	17.0%
Op. income	(236)	57	(250)	225
Adj. EBITDA	(45)	199	(130)	345
Diluted EPS	\$(1.22)	\$0.10	\$(1.13)	\$0.72
Members (000)	1,036	1,677	1,880	1,400
BVPS	\$3.51	\$4.05	\$3.41	\$4.47

FY23A and FY24A are company actuals per the 7 Feb 2024 and 4 Feb 2025 results releases (SEC EDGAR). FY25E revenue, MLR, SG&A ratio and operating loss are the midpoints of guidance revised on 22 July 2025 and reaffirmed on 6 August 2025 (\$12.0bn to \$12.2bn revenue, 86.0% to 87.0% MLR, 17.1% to 17.6% SG&A, loss from operations of \$200m to \$300m, adjusted EBITDA loss ~\$120m smaller than the operating loss). FY25E EPS, FY25E year-end membership and book value, and all FY26E lines are analyst-derived: the company has issued no 2026 guidance beyond a stated intent to return to profitability. Further sourcing detail is set out under Important Disclosures.

The concentration is the point and the risk. Substantially all of Oscar's premium comes from one federally subsidised market whose economics are set annually by state rate review, by the CMS risk adjustment transfer formula and by the size of the federal premium tax credit. Oscar has no Medicaid or commercial group ballast; it exited Medicare Advantage for 2025.

Q2 2025 RESULTS AND THE JULY GUIDANCE CUT

Oscar pre-announced on 22 July and reported in full on 6 August. Q2 revenue of \$2,864m rose 29% year on year on premiums of \$2,803m, but the medical loss ratio reached 91.1% against 79.0% in Q2 2024 and the SG&A ratio was 18.7%. The result was a loss from operations of \$230.5m, a net loss of \$228.4m, or \$(0.89) per diluted share, and an adjusted EBITDA loss of \$199.4m. Consensus had been \$(0.81) on \$2,919m of revenue, so the quarter missed on both lines. First-half figures remain positive: revenue of \$5,910m, an 83.0% MLR, earnings from operations of \$66.6m and net income of \$46.9m, or \$0.17 per share. All of the damage is in one quarter, and within that quarter in one accrual.

The revised guidance is the more important disclosure. On 4 February management had guided 2025 to revenue of \$11.2bn to \$11.3bn, an MLR of 80.7% to 81.7%, an SG&A ratio of 17.6% to 18.1% and earnings from operations of \$225m to \$275m, and reaffirmed all of it on 7 May. The 22 July revision moved revenue up to \$12.0bn to \$12.2bn while moving the MLR to 86.0% to 87.0% and earnings from operations to a loss of \$200m to \$300m, with adjusted EBITDA loss running roughly \$120m smaller than the operating loss. Revenue rose and profit collapsed: membership and price came in ahead of plan, and the cost of that membership came in far worse. On the midpoints that is a \$500m deterioration in operating profit and, on our arithmetic, a full-year net loss of roughly \$290m against a \$25m profit in 2024.

The trigger was external and dated. Oscar cited a review of second-quarter marketplace risk adjustment data from Wakely, incorporating claims through 30 April, which showed ACA marketplace risk scores rising by more than the company had assumed. Bertolini quantified it as morbidity "increasing by mid to high single digits across Oscar's markets". Centene had published the same signal three weeks earlier, withdrawing 2025 guidance on 1 July after Wakely data covering 22 of its 29 marketplace states and roughly 72% of its marketplace membership implied a \$1.8bn reduction to net risk adjustment revenue, about \$2.75 per share. This is a market-wide event, not an Oscar underwriting failure, which is both the mitigant and the problem: Oscar cannot outrun a pool it does not control.

We take two things from the guide. First, the implied second half is worse than the headline. On our arithmetic the full-year MLR midpoint of 86.5% against a first-half 83.0% implies a second-half MLR of roughly 90%, against roughly 86% in the second half of 2024. Second, management's forward statement is deliberately unquantified. Bertolini said only that "we believe the market will stabilize next year, and we expect to return to profitability in 2026", against a long-term margin target of 5%. There is no 2026 revenue, membership, MLR or EPS guidance. An investor buying the shares at \$17.02 is underwriting a number the company has declined to name.

FINANCIAL SUMMARY

Oscar Health, Inc. -- Summary Financials and Operating Metrics

\$ in millions, except per-share data	FY23A	FY24A	FY25E	FY26E	FY27E
Total revenue	5,863	9,178	12,100	11,250	12,200
% growth YoY	48%	57%	32%	-7%	8%
Premiums earned	5,686	8,971	11,870	11,040	11,970
Medical loss ratio	81.6%	81.7%	86.5%	81.0%	80.0%
SG&A expense ratio	24.3%	19.1%	17.4%	17.0%	16.5%
Earnings/(loss) from operations	(236)	57	(250)	225	427
Operating margin	-4.0%	0.6%	-2.1%	2.0%	3.5%
Adjusted EBITDA	(45)	199	(130)	345	552
Net income/(loss)	(271)	25	(290)	190	385
Diluted EPS	\$(1.22)	\$0.10	\$(1.13)	\$0.72	\$1.42
FY23A/FY24A per company disclosure; FY25E guidance midpoints; FY26E/FY27E analyst-derived					
Members, period end (000)	1,036	1,677	1,880	1,400	1,470
Book value per share	\$3.51	\$4.05	\$3.41	\$4.47	\$6.21
Memo: P/E at \$17.02	n.m.	n.m.	n.m.	23.6x	12.0x
Memo: P/B at \$17.02	4.85x	4.20x	4.99x	3.81x	2.74x

Source: Oscar Health results releases of 7 February 2024, 4 February 2025, 7 May 2025, 22 July 2025 and 6 August 2025 (SEC EDGAR and company IR); Q2 2025 earnings call transcript, 6 August 2025; balance-sheet data via stockanalysis.com; Akul Veauli Securities LLC. FY25E revenue, MLR, SG&A ratio and operating loss are guidance midpoints; FY25E premiums earned, adjusted EBITDA, net loss, EPS, year-end membership and book value are analyst-derived from that guidance (premiums earned held at 98.1% of total revenue, the 1H25 ratio; below-the-line expense annualised from the \$19.7m 1H25 gap between operating and net income; adjusted EBITDA taken as \$120m better than the operating loss per the 22 July release). FY26E and FY27E are entirely analyst-derived and assume the enhanced premium tax credits lapse on 31 December 2025; the company has issued no 2026 or 2027 guidance. Multiples are computed on the \$17.02 close of 27 August 2025 and 258.2m shares outstanding at 30 June 2025, with forward-period share counts of 264m (FY26E) and 271m (FY27E) reflecting stock compensation dilution. "n.m." denotes a loss-making or otherwise meaningless multiple.

THE RISK ADJUSTMENT TRUE-UP: ONE LINE ITEM, NOT A TREND BREAK

Investors reading Oscar alongside the Medicare Advantage names should be careful not to blend two different problems into a single "utilisation is running hot" narrative. In Medicare Advantage the 2025 shock has been driven by care consumption and by coding under the V28 model. In the ACA individual market the dominant driver in Q2 was the composition of the risk pool, expressed through the CMS risk adjustment transfer, and the two are not the same variable.

The arithmetic is close to decisive. Blackley disclosed "an incremental \$316 million increase to our risk adjustment payable for 2025" recorded in the second quarter. On \$2,803m of Q2 premium that single accrual is 11.3 percentage points. The reported MLR deteriorated by 12.1 points year on year, from 79.0% to 91.1%. Grossing the accrual back out leaves an underlying Q2 MLR of roughly 79.8%, against 79.0% a year earlier: eighty basis points of genuine cost deterioration, not twelve hundred. This is a simple gross-up rather than a restatement, and the accrual is a real 2025 cost that will recur while morbidity stays elevated, so we do not present 79.8% as the true run rate. But it does establish where the pressure is coming from. Blackley's own characterisation was that "second quarter utilization moderated meaningfully as compared to the first quarter" and that the company "saw sequential softening in utilization each month in the second quarter".

Why the pool worsened is well understood by the company. Bertolini attributed it to "consumers entering the individual market for Medicaid redeterminations and healthier, low-utilizing consumers leaving the market", the classic adverse-selection spiral that the enhanced subsidies were designed to suppress. Two policy changes tighten it further from here. Continuous monthly special enrolment for consumers at or below 150% of the federal poverty level ended on 1 September 2025, which management expects to moderate second-half additions; and the CMS Marketplace Integrity and Affordability rule finalised in June 2025 adds eligibility verification friction that falls hardest on the low-utilising marginal enrollee.

The read-through is genuinely two-sided and we will not pretend otherwise. A morbidity problem is fixable with price in a market that reprices annually, which is why we do not treat 2025 as an impairment of the franchise. But it is also a problem Oscar shares with every carrier in the pool and cannot underwrite its way out of, and it arrives in the same year the subsidy that holds the pool together is scheduled to lapse. Pricing corrects a level; it does not correct a composition that keeps moving against you.

2026: THE SUBSIDY CLIFF AND THE REPRICING ARITHMETIC

The enhanced premium tax credits enacted in 2021 and extended by the Inflation Reduction Act expire on 31 December 2025. As of this note nothing has been enacted to extend them. Marketplace enrolment reached a record 24 million in 2025 under those credits, roughly double the pre-2021 level; the Congressional Budget Office has estimated that expiry alone leaves about 4.2 million more people uninsured. Bertolini told investors on 6 August that Oscar sees "potential upside with growing support to renew enhanced premium tax credits in the upcoming continuing resolution". That is a real possibility and we treat it as the principal upside case, but it is a hope, not a fact, and Oscar's own guidance assumes expiry.

Price is moving at the same time. The Peterson-KFF analysis published 6 August 2025 found a median proposed 2026 increase of 18% across 312 marketplace insurers, an average near 20%, with an interquartile range of 12% to 27% and outliers from -10% to +59%. Insurers assuming subsidy expiry added roughly four points for the resulting morbidity shift, on top of underlying medical trend of around 8% and, for some, roughly three points for tariffs. Oscar's Pennsylvania subsidiary filed 22.0% on 15 May; management then confirmed on 6 August that it had "resubmitted 2026 rate filings in states covering nearly all current membership to reflect morbidity increases", with Blackley saying increases would be "multiple times what we saw last year". Oscar has not published a company-average filed increase and we do not assign one; we model 24%, inside the market interquartile range and above the median.

Put the two together and 2026 revenue falls. We assume average membership declines roughly 25% from about 1.97m in 2025 to about 1.48m, and revenue per member rises roughly 24%, giving total revenue of \$11.25bn, a 7% decline. On the cost line the mechanical bridge is favourable: a 24% rate increase against effective claims trend of roughly 13% (8% medical trend plus the morbidity load) takes the 86.5% guided MLR to roughly 78.6%. We do not use that number. We model 81.0%, allowing two full points for adverse selection running worse than the actuarial load, which is what has happened in every previous subsidy contraction. With SG&A at 17.0%, helped by per-member costs falling faster than revenue, that is a 2.0% operating margin, \$225m of operating income and \$0.72 of EPS. It is a return to profitability. It is also less than half the 5% margin management describes as the long-term target, and it arrives on a revenue base that has shrunk.

VALUATION AND PRICE TARGET

Our \$15.50 Dec-27 target is the equal-weighted average of three mechanical anchors applied to FY27E, the first year in which we assume the repricing has fully seasoned. (1) Forward earnings: 12.0x FY27E EPS of \$1.42 gives \$17.04 (10x: \$14.20; 15x: \$21.30). Twelve times is a premium to where the government-programme insurers were trading through the 2025 de-rating and a discount to the market, sized to Oscar's growth optionality against its sub-scale, single-market risk. (2) Book value: 2.50x FY27E book value per share of \$6.21 gives \$15.53 (2.0x: \$12.42; 3.0x: \$18.63). The book value path runs from \$4.49 at 30 June 2025 down to \$3.41 at year-end 2025 on the guided second-half loss, then up to \$4.47 and \$6.21 as earnings return; 2.50x is a modest premium to Molina's 2.05x, justified by an implied 2027 return on equity near 27%. (3) Revenue: 0.30x FY27E revenue of \$12.2bn on 271m shares gives \$13.46 (0.25x: \$11.21; 0.40x: \$17.94), against Molina at 0.233x and Centene at 0.087x on FY24 revenue. The simple average is \$15.34, rounded to \$15.50, an implied return of -8.9% with no dividend, which maps to Underweight under our above +15% and below 0% thresholds. At target Oscar would trade on 10.9x FY27E EPS, 2.50x forward book, 0.35x FY27E revenue and a \$4.2bn market capitalisation.

We should be explicit about how close this is and what moves it. Neutral requires a target of \$17.02, roughly 11% above our blend; Overweight requires \$19.57. Taking the top of every multiple range simultaneously, 15.0x, 3.0x and 0.40x, produces \$19.29, an implied +13.3%, which is still Neutral. In other words, on the base-case earnings path there is no combination of defensible multiples that gets to Overweight. The rating is a function of the earnings path, not of multiple choice, and the earnings path is a function of one legislative outcome.

That is the steel-man, and it is a good one. If the enhanced credits are extended before open enrolment, membership holds near 1.95m, 2026 revenue lands near \$14.8bn on rates already filed, and a 3.0% margin gives roughly \$1.50 of 2026 EPS. Carrying that into 2027 at \$15.3bn of revenue and a 4.0% margin gives EPS of about \$2.03 and book value per share of about \$7.51. The same three anchors then produce \$24.36, \$18.78 and \$16.63, a blend of \$19.92 and an implied +17.0%, which would be Overweight. An investor who assigns better than roughly even odds to extension should own the shares, and that is a respectable position. We do not, because nothing has been enacted, because carriers have already filed and in many cases locked 2026 rates on the assumption of expiry, and because a late extension would create its own disruption rather than simply restoring the status quo.

The relative-value evidence is what tips us. Oscar trades at 3.79x June book against Centene at 0.52x and Molina at 2.05x on the same 27 August closes, and at 0.36x FY25E revenue against Molina at 0.23x and Centene at 0.09x on FY24 revenue. Those peers are impaired, and Oscar deserves to trade above them: it grows, it has \$5.38bn of cash and investments against \$357m of debt, and its insurance subsidiaries held roughly \$1.2bn of capital and surplus including \$579m of excess capital at 30 June. But a franchise whose best-ever operating margin is the 0.6% it earned in 2024, which is now guiding to a \$250m operating loss, does not obviously deserve to trade at seven times Centene's book multiple. The shares have already risen 32% from the \$12.86 intraday low struck the week of 21 July, and they rose in the week Oscar cut guidance. The recovery is not being anticipated. It is being paid for.

UPSIDE RISKS

Risks that would drive the shares above our price target include, but are not limited to, the following:

- Enhanced premium tax credits are extended. This is the whole call. On our own arithmetic extension lifts the three-anchor blend to \$19.92, an implied +17.0%, which would be an Overweight rating rather than an Underweight one.
- The repricing overshoots. A 24% rate increase against 13% effective claims trend mechanically implies a 78.6% MLR, not the 81.0% we model. Delivering the mechanical number adds roughly 240bp of margin and takes FY26E EPS toward \$1.40.
- Utilisation is already improving. Management reported sequential monthly softening through Q2 and characterised the moderation as meaningful. If that persists, the second-half MLR runs below the roughly 90% the guidance implies and full-year losses come in under \$200m.
- Membership attrition is smaller than feared. Higher gross premiums fall mostly on the subsidised, and the unsubsidised and ICHRA channels are growing. A 15% rather than 25% decline adds roughly \$1.4bn of 2026 revenue and around \$30m of operating income at target margins.
- Competitor exit is the underrated offset. Carriers withdrawing from marginal counties for 2026 leave share for those that stay, and Oscar's stated position is that it is pricing to persist. Share gains at repriced rates are the highest-quality growth available.
- Capital is not the constraint. \$5.38bn of cash and investments, \$579m of excess subsidiary capital and only \$357m of debt mean Oscar can fund a bad 2026 without an equity raise, removing the dilution risk that usually accompanies a loss year.
- The stock is a high-beta option on one binary. A \$11.20 to \$23.79 52-week range and roughly 19m shares of daily volume mean that a credible extension headline can reprice the equity faster than any fundamental revision we would make.

DOWNSIDE RISKS

Risks that would drive the shares below our price target include, but are not limited to, the following:

- The subsidy cliff arrives as scheduled and enrolment falls harder than modelled. Marketplace enrolment doubled to 24 million under the enhanced credits; the CBO estimates expiry alone leaves roughly 4.2 million more people uninsured. A 35% rather than 25% membership decline takes 2026 revenue below \$10bn and, on fixed-cost deleverage, likely back to an operating loss.
- Single-market concentration with no ballast. Substantially all premium comes from the ACA individual exchanges. Oscar has no Medicaid or commercial group book to absorb a bad marketplace year, unlike every diversified peer it is compared to.
- Adverse selection is a moving target, not a level. The 2025 morbidity shock arrived after pricing was locked. If the healthier residual leaves faster than the four-point actuarial load assumes, 2026 repeats 2025 with the correction one year behind the deterioration.
- Risk adjustment is a transfer Oscar does not control. The \$316m second-quarter accrual was driven by other carriers' risk scores, not Oscar's own claims. The formula can move against the company again on data it sees only twice a year, and the June and 2Q Wakely cycles are the disclosure events to watch.
- Valuation carries the burden. 3.79x June book against Centene at 0.52x and Molina at 2.05x, and 0.36x FY25E revenue against 0.23x and 0.09x. A de-rating to Molina's 2.05x book on our FY26E book value of \$4.47 implies roughly \$9.20.
- No quantified 2026 guidance. Management has committed only to "return to profitability in 2026" against a 5% long-term margin target it has never achieved; the best operating margin in company history is the 0.6% earned in 2024. Investors are underwriting an unnamed number.
- Second-half 2025 is worse than the headline. The guided full-year 86.5% MLR against a first-half 83.0% implies roughly 90% in the second half, against roughly 86% a year earlier, and management flagged possible incremental fourth-quarter utilisation if members pull care forward ahead of subsidy expiry.
- Regulatory tightening compounds the cliff. The CMS Marketplace Integrity and Affordability rule and the 1 September end of continuous monthly special enrolment below 150% of the federal poverty level both remove low-utilising members, and management sized the dual-eligible cohort alone at roughly 2.5% of membership, just under 50,000 lives.
- Book value is falling before it rises. The guided second-half loss takes book value per share from \$4.49 at 30 June to roughly \$3.41 at year-end on our estimates, so the P/B multiple gets worse before the earnings recovery makes it better.

ANALYST CERTIFICATION

I, Akul Veauli, hereby certify that the views expressed in this research report accurately reflect my personal views about the subject company and its securities. I also certify that no part of my compensation was, is, or will be directly or indirectly related to the specific recommendations or views expressed in this report. This report is a template and style exercise; the named individual and firm are fictitious and this certification is illustrative only.

IMPORTANT DISCLOSURES

Estimate and market-data sourcing, continued from the cover page. FY26E assumes the enhanced premium tax credits lapse on 31 December 2025, average membership down ~25% and revenue per member up ~24%. Market data as of the 27 August 2025 close. Short interest and beta dated on or before that close were not obtainable from an unpaywalled source and are shown as "-".

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Estimate sourcing and derivation: the \$15.50 price target is the equal-weighted average of (i) 12.0x analyst-derived FY2027E EPS of \$1.42, (ii) 2.50x analyst-derived FY2027E book value per share of \$6.21 and (iii) 0.30x analyst-derived FY2027E revenue of \$12.2bn on 271m shares. The rating rule was stated in advance: Overweight above +15% implied twelve-month total return, Underweight below 0%, Neutral otherwise. FY2025E figures are the midpoints of guidance revised on 22 July 2025 and reaffirmed on 6 August 2025; FY2025E EPS, book value and year-end membership are analyst-derived from that guidance. FY2026E and FY2027E are entirely analyst-derived and assume the enhanced ACA premium tax credits lapse on 31 December 2025. The company has published no 2026 or 2027 guidance beyond a stated intention to return to profitability in 2026 and a long-term margin target of 5%. Street consensus estimates for FY2025 through FY2027 that could be reliably dated on or before 27 August 2025 were not obtainable from an unpaywalled source and are therefore not presented; the only dated consensus figures used are the Q2 2025 estimates of \$(0.81) EPS on \$2,918.6m of revenue reported alongside the 6 August print. Short interest and beta dated on or before the 27 August 2025 close were likewise not obtainable from an unpaywalled source and are shown as "-" rather than estimated. The underlying Q2 2025 medical loss ratio of roughly 79.8% cited on page 3 is a simple gross-up of the disclosed \$316m risk adjustment accrual against Q2 premiums earned and is not a company-reported or restated figure.

Market data, including the \$17.02 close of 27 August 2025, the \$11.20 to \$23.79 52-week intraday range, the 19.2m average daily volume across the nineteen August 2025 sessions to 27 August, and the Centene and Molina closes of \$28.84 and \$176.12 used in the peer comparison, are as of the 27 August 2025 close. The price performance chart on page 1 plots actual weekly closing prices from 28 August 2024 to 27 August 2025 and contains no data after the as-of date.

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RATING DEFINITIONS

Overweight: total return expected to exceed the average total return of the analyst's coverage universe over the next 6-12 months. Neutral: total return expected to be broadly in line with that average. Underweight: total return expected to be below that average. For this note the rating was applied mechanically to the implied twelve-month total return from the derived price target: Overweight above +15%, Underweight below 0%, Neutral otherwise. OSC's implied return of -8.9%, being the \$15.50 target against the \$17.02 close of 27 August 2025 with no dividend, maps to Underweight. The flip thresholds are stated in the Valuation section: Neutral requires a target of \$17.02 and Overweight a target of \$19.57. These definitions are illustrative, generic sell-side rating conventions provided for template purposes only.

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